

When to Call Your Provider

Clear guidance on heavy bleeding, severe pain, dehydration, fever, or signs of ectopic pregnancy.

Navigating the medical system during the first trimester requires a shift in mindset. You are accustomed to powering through discomfort or waiting out a flu, but pregnancy changes the threshold for seeking care. You are not "bothering" your doctor or midwife by reporting a concern; you are providing them with essential data to manage your health and the pregnancy safely. If you experience any of the following, do not wait for your next scheduled appointment.

Severe Pain and Ectopic Warning Signs

While mild uterine stretching is standard, pain that stops you in your tracks is not. The most critical condition to rule out in early pregnancy is an ectopic pregnancy—where the embryo implants outside the uterus, typically in the fallopian tube. This is a medical emergency. Seek immediate care if you experience:



- **Sharp, unilateral pain:** Severe stabbing pain on one specific side of your lower abdomen.
- **Shoulder tip pain:** A strange, distinct pain where your shoulder meets your arm. This is a referred nerve pain caused by internal bleeding irritating the diaphragm.
- **Fainting or severe dizziness:** A sign of a drop in blood pressure.

Uncontrollable Vomiting (Hyperemesis)

There is a clinical difference between standard morning sickness and *Hyperemesis Gravidarum* (HG). Standard nausea is miserable but manageable; HG is dangerous due to rapid dehydration and nutrient depletion.



Call your provider if:

- You cannot keep down fluids for 24 hours.
- You show signs of severe dehydration, such as very dark urine, urinating infrequently (less than every 6–8 hours), or feeling dizzy when standing.

You have lost more than 5% of your pre-pregnancy body weight.

Intervention with IV fluids and prescription anti-nausea medication is often necessary and safe.